

SUBJECTIVE:

- Patient presents with complaints of feeling "very winded" which they localize to their lungs, accompanied by a subjective pain score of 7.8 out of 10.
- The patient also reports a "slight chest pain" specifically when taking a deep breath.
- These symptoms reportedly began approximately one day prior, following a fall where the patient directly struck their sternum on a "pointed staff."
- The impact site is described as "right in the sternum."

OBJECTIVE:

- Expected physical exam findings include tenderness to palpation directly over the sternum, with potential for crepitus or a palpable step-off if a fracture is present.
- Lung auscultation should be performed to assess for clear breath sounds or any diminished sounds that could indicate a pneumothorax.
- Respiratory rate and effort should be noted for signs of distress. Oxygen saturation monitoring is crucial.
- Cardiovascular examination for muffled heart sounds or arrhythmias would be relevant in high-energy impacts.
- Vital signs: Unavailable.

ASSESSMENT:

- Primary Diagnosis: Acute Sternal Contusion versus Sternal Fracture. The mechanism of injury (direct impact to sternum) and subjective symptoms of localized pain (especially pleuritic with deep inspiration) and dyspnea are highly suggestive of a musculoskeletal injury to the chest wall. The patient's reported pain level of 7.8 and feeling "very winded" elevate concern beyond a simple contusion.
- Differential Diagnoses:
 - Rib Fractures: While the patient indicates sternal pain, associated or isolated rib fractures cannot be definitively ruled out without imaging, especially with a fall mechanism.
 - Pneumothorax: Due to the complaint of feeling "very winded" and chest pain, pneumothorax must be considered with any chest trauma, despite the sternal localization.
 - Myocardial Contusion: A rare but serious complication of blunt sternal trauma, particularly with high-energy impacts, requiring consideration.

PLAN:

- Tests:
 - Chest X-ray (CXR): To evaluate for sternal fracture, rib fractures, pneumothorax, or other pulmonary abnormalities.
 - Electrocardiogram (ECG): To rule out myocardial contusion, especially if high-energy impact or if cardiac symptoms emerge.
 - Complete Blood Count (CBC) and Basic Metabolic Panel (BMP): If clinically indicated to assess for general health or electrolyte imbalances.
- Medications:
 - Over-the-counter (OTC) non-steroidal anti-inflammatory drugs (NSAIDs) such as ibuprofen, or acetaminophen, for pain management.
 - Prescription analgesics may be considered if OTC options prove insufficient for pain control.
- Patient Advice:
 - Apply ice packs to the sternal area for 15-20 minutes several times a day to help reduce pain and swelling.
 - Avoid strenuous activities, heavy lifting, and any movements that exacerbate chest pain.
 - Encourage gentle deep breathing and controlled coughing to prevent lung complications, while being mindful of pain.
 - Advise patient to return immediately if they experience worsening shortness of breath, severe uncontrolled chest pain, dizziness, lightheadedness, or any signs of severe distress.
- Follow-up Instructions:
 - Follow-up with primary care provider or an urgent care clinic within 24-48 hours to review imaging results and reassess symptoms.